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Patient Name: **Mrs. DHANMANTI**	PateintID : **LDPL/00657681**
Age/Gender : 52 Y/Female	Visit No : DEL2605125408
Refer Client : LDPL9269 ZAHEED KHAN	Collected On : 12/May/2026 10:49 PM
Refered By : N/A	Received On : 12/May/2026 10:51 PM
Doctor Name:	Reported Date12/May/2026 11:58 PM
SampleType : Whole Blood EDTA - 21926710	

HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
COMPLETE BLOOD COUNT (CBC),WHOLE BLOOD EDTA			
HAEMOGLOBIN (Hb) Colorimetric method	**10.6**	g/dL	12.0-15.0
RED BLOOD CELLS- RBC COUNT DC Impedance with hydrodynamic focusing	**3.76**	millions/mm³	4.5 - 5.5
PACKED CELL VOLUME (PCV) - HEMATOCRIT Pulse Height detection method	**30.4**	%	40.0-50.0
MCV Automated/Calculated	**80.9**	fL	83-101
MCH Automated/Calculated	28.1	pg	27.0-32.0
MCHC Automated/Calculated	**34.9**	g/dL	31.5-34.5
RED CELL DISTRIBUTION WIDTH (RDW-CV) Automated/Calculated	**17.5**	%	11.6-14.0
RED CELL DISTRIBUTION WIDTH (RDW-SD) Automated/Calculated	**54.4**	fL	39.0- 46.0
MENTZER INDEX Calculated	21.52		
PLATELET COUNT DC Impedance with hydrodynamic focusing/	337	10^3/µL	150-410
PLATELET DISTRIBUTION WIDTH (PDW) Calculated	16.2	fL	9.00-17.00
PCT(PLATELETCRIT) Calculated	**0.291**	%	0.108-0.282
MEAN PLATELET VOLUME - MPV Calculated	8.6	fL	7.00-12.0
P-LCR Calculated	17.20	%	11.0-45.0
P-LCC Calculated	58.00	10^9/L	30.0-90.0
TOTAL LEUKOCYTE COUNT (TLC) Electrical impedance	4.88	10^3/µL	4.00-10.0
DIFFERENTIAL LEUCOCYTE COUNT			
Neutrophils Flow cytometry/Manual	74.0	%	40 - 80
Lymphocytes Flow cytometry/Manual	**11.6**	%	20 - 40
Eosinophils Flow cytometry/Manual	2.8	%	1.00-6.00
Monocytes Flow cytometry/Manual	**11.4**	%	2.00-10.0

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HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
Basophils Flow cytometry/Manual	0.2	%	0.00-1.00
ABSOLUTE NEUTROPHIL COUNT Calculated	3.60	10^3/µL	2.00-7.00
ABSOLUTE LYMPHOCYTE COUNT Calculated	**0.57**	10^3/µL	1.00-3.00
ABSOLUTE EOSINOPHIL COUNT Calculated	0.14	10^3/µL	0.02-0.50
ABSOLUTE MONOCYTE COUNT Calculated	0.56	10^3/µL	0.20-1.00
ABSOLUTE BASOPHIL COUNT Calculated	0.01	10^3/µL	0.02-0.10

CLINICAL NOTES
A complete blood count (CBC) is used to evaluate overall health and detect wide range of disorders, including anemia, infection and leukemia.There have been some reports of WBC and platelet counts being lower in venous blood than in capillary blood samples ,although still within these reference ranges.

POSSIBLE CAUSES OF ABNORMAL PARAMETERS	
High RBC, Hb, or HCT - dehydration, polycythemia,shock, chronic hypoxia	**Low RBC, Hb, or HCT -** anemia, thalassemia, and other hemoglobinopathies.
High MCV - macrocytic anemia, liver disease	**Low MCV -** microcytic anemia.
High WBC -acute stress, infection,malignancies	**Low WBC -** sepsis, marrow hypoplasia
High platelets - risk of thrombosis	**Low platelets -** risk of bleeding

Notes
1.Macrocytic Anemia/Dimorphic Anemia can have low platelet count.
2.Microcytic Anemia/Leucocytosis can have Reactive thrombocytosis.

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HAEMATOLOGY

Test Name	Result	Unit	Bio. Ref. Interval
ERYTHROCYTE SEDIMENTATION RATE (ESR),WHOLE BLOOD EDTA			
ESR [WESTERGREN] Sedimentation	**26**	mm/1st	0 - 15

CLINICAL NOTES
The erythrocyte sedimentation rate (ESR) is a relatively simple, inexpensive, non-specific test that has been used for many years to help detect inflammation associated with conditions such as infections, cancers, and autoimmune diseases.This test may also be used to monitor disease activity and response to therapy in both of the above diseases as well as some others, such as lupus.

Factors increasing ESR	Factors decreasing ESR
Old age,Pregnancy,Anemia	Microcytosis
Elevated fibrinogen	Low fibrinogen
Macrocytosis	Polycythemia
Macrocytosis	Marked leukocytosis

End of Report

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